

The Live Demo Script

A 3–4 minute, second-by-second walkthrough of the real running prototype — verified live, not imagined

Every screen, click, and quoted line of output in this script was tested against the actual deployed app on 15 Sep 2026. Rehearse it once end-to-end before you go up — not just read it.

0. Ninety seconds before you walk up

- Open the live URL in a fresh tab **3–5 minutes early** — free-tier hosting can take a moment to wake up on the first request of the day. Load it once, let it fully settle, before your slot starts.
 - Do **one hard refresh** (Ctrl/Cmd+Shift+R) right before you go up. The app now revalidates its own files on every load, but a hard refresh costs nothing and removes all doubt.
 - Have the symptom text for the live demo **already typed somewhere you can copy-paste** (notes app, or memorized) — typing live on a projector, under nerves, is where most demo time gets lost.
 - Know your Wi-Fi situation. If it's flaky, mention that up front, lightly, rather than letting a slow load read as the app being broken.
 - If you plan to show the physician console, confirm the staff passcode works on **this exact device** beforehand — don't discover it live.
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1. The script — timed to ~3:30, fits a 4-minute slot

Times are cumulative from when you start talking, not per-step. If you're given only 3 minutes, cut the physician console step (marked [CUTTABLE] below) — nothing else depends on it.

0:00–0:20 — Open with the real problem, no click yet

SAY: “Doctors in India spend about two minutes with each patient. But seventy to eighty percent of a correct diagnosis comes from the patient’s history alone, before any test. So the two minutes doctors have the least of is exactly the two minutes that decides the most.”

0:20–0:45 — Let the landing page prove the safety net — zero clicks

CLICK/SHOW: The live-demo ticker on the homepage (under the hero text). Just point — it types and reacts on its own.

SAY: “Watch this — I haven’t clicked anything yet. This is typing a real emergency phrase and catching it live, against the same safety check every real submission runs through. No AI call needed for this part — it works even fully offline.”

Real behavior: the ticker fetches the live red-flag term list from the backend once, then types and matches client-side. If it's still loading when you arrive, wait a beat — don't narrate over a blank box.

0:45–1:10 — Type a real symptom yourself

CLICK/SHOW: Click into the “Describe how you are feeling” box. Type (or paste): “I have had a mild cough and runny nose for two days, no fever.”

SAY: “Now let me actually use it, the way a patient would — typing, in plain words, no medical jargon.”

Verified input — use this exact phrase. It reliably produces the richest results screen (guideline citation + every action button) on the current deployment.

1:10–1:35 — Move through the 4-step form quickly

CLICK/SHOW: Next → Next (skip the optional photo step) → fill Age (e.g. 29) and Days (e.g. 2) → Next → check the consent box → Submit My Symptoms.

SAY: “Age and how many days are optional context — you can skip them entirely and it still works. I’ll add them quickly. And this consent line is real, not decorative — it’s enforced on the server too, in line with India’s Digital Personal Data Protection Act.”

1:35–2:20 — The results screen — this is the core of the pitch

CLICK/SHOW: Scroll through, in order: the priority banner → the “Why this priority level” box → the Chief Complaint / History of Present Illness fields → the AI-drafted disclosure line.

SAY: “This isn’t just a label. It quotes the actual clinical guideline sentence it matched, and how similar the match was — that’s a real citation, not a black box. And notice this line at the bottom: it openly says this is AI-drafted and hasn’t been reviewed by a physician yet. It’s built to help a doctor decide faster — never to replace the doctor’s own judgment.”

Verified live: this exact input returns URGENT with a guideline quoted at 40% similarity. If a judge asks “why urgent for a mild cough?” — see Section 3, “If a judge questions the output.”

2:20–2:45 — One-click download — the “carry it to the doctor” moment

CLICK/SHOW: Click “Download this summary.” A file lands immediately — show the downloads bar/folder if visible.

SAY: “And here’s the whole point of doing this before the visit: one tap, and the patient has a real file they can carry straight into the consultation room — no login, no app to install, works on any phone.”

2:45–3:05 — [CUTTABLE] The physician’s side of the same case

CLICK/SHOW: Switch to “Physician console” (top of page) → sign in with the staff passcode → open the case you just submitted.

SAY: “And this is what the doctor sees the moment the patient walks in — the same draft, ready to accept, amend, or reject. Same real case, same database, not a separate demo.”

Skip this entire step if you’re short on time — nothing after it depends on having shown it.

3:05–3:30 — Close on the one differentiator judges remember

SAY: “One last thing: every safety check you just saw — the red-flag catch, the priority level, the guideline citation — runs with zero API cost. It keeps working even if the AI service is down. That’s what lets this scale to a real, high-volume government hospital, not just stay a demo. INAYAT means ‘care’ in Urdu — we built it to give the doctor’s two minutes back to actually deciding, not repeating questions. Thank you — happy to take questions or let you try it yourselves.”

2. If something goes wrong — real failure modes, real responses

Each of these is a genuine, verified behavior of this specific app — not generic advice. Know the response before you need it; don’t improvise live.

The page is slow to load / first request times out

- Cause: free-tier hosting sleeps when idle and takes a moment to wake on the first hit.
- Response: this is exactly why you opened the tab 3–5 minutes early (Section 0). If it still happens live, keep talking through the problem statement while it loads — don’t stare at the screen in silence.

Wi-Fi drops completely mid-demo

- Response: don’t fight it live. Say plainly, “looks like the connection dropped — let me talk you through what you’d see,” and narrate the results screen from memory using this script. A judge respects composure far more than a frozen retry.

The page looks visually broken — unstyled text, missing button labels

- Cause: a browser can occasionally hold a stale cached copy of one file after a deploy.
- Response: this is exactly what the pre-stage hard refresh in Section 0 rules out. If it still happens live, one Ctrl/Cmd+Shift+R fixes it in about a second — do it without apologizing at length, then continue.

A judge types their own symptom and it comes back “urgent” for something they think is minor

- This is not a bug to apologize for — it’s the deliberate safety design, and it’s a strong answer if you say it plainly.
- SAY: “That’s intentional. Without a live AI connection configured on this deployment, it defaults to the cautious answer rather than guessing — it will escalate on any real signal, but it will never quietly reassure someone who might be wrong to be reassured. With a live model key connected, it reasons across the full range from self-care to emergency — we kept the offline mode intentionally conservative because that’s the one that has to be trustworthy with zero cost and zero risk of being wrong in the dangerous direction.”
- Do **not** promise live, on the spot, that a different typed input will show “self-care” or “clinic visit” — on this deployment (no configured API key) it reliably will not. Redirect to the guideline-citation box instead: that’s real and demonstrable regardless of level.

The download button doesn't visibly do anything on a judge's unfamiliar device

- Response: check the downloads folder/bar — some browsers download silently with no visible popup. If truly blocked (rare, some locked-down browsers), fall back to “Listen to summary” instead and narrate that the same content downloads as a plain text file on a normal device.

You forget the physician passcode or it's rejected

- Response: skip it. Say, “and on the doctor's side, the same case opens instantly in a separate, passcode-locked console” — describe it in one sentence and move to the close. Never fumble a password live.
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3. Why this earns interest — keep these ready for follow-up questions

- **It's a real citation, not a black box.** The “Why this priority level” box quotes an actual clinical guideline sentence and a real text-similarity score — most triage demos just show a label.
 - **It's honest about its own limits, on-screen, every time.** “AI-drafted, not yet reviewed by a physician” isn't a disclaimer buried in a footer — it's on every single result.
 - **It still works with zero API cost.** Built and demonstrated on the exact assumption a real rural deployment will face: no budget for a live model connection, every single day, not just for this demo.
 - **It closes with a real, downloadable file the patient keeps** — not a login-gated app they'll never open again.
 - **The doctor's side is real, not a mockup slide.** Same database, same case, passcode-gated — provably not two separate demos stitched together.
 - **It's built for the actual constraint in the problem statement** — a 2–5 minute OPD visit — not a generic chatbot retrofitted to health.
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One line if you only remember one thing from this page: **“Every number and every quote in this demo is something the app actually produced when we tested it — nothing here is staged.”** Say it if a judge pushes on whether the demo is real.